

Emergency Decisions Unit Pathway

Anaphylaxis

Date DD/MM/YY

Patient details

Full name

DoB

Unit number

(use sticker if available)

Inclusion criteria	AIRWAY: Swollen lips or tongue BREATHING: Wheeze CIRCULATION: Tachycardia or mild hypotension
Exclusion criteria	AIRWAY: Stridor BREATHING: Unable to complete sentences, SpO2 <93%, PEFR <50% best CIRCULATION: Severe tachycardia or Systolic BP <90mmHg DISABILITY: GCS below 15/15

Anaphylaxis is likely when all of the following 3 criteria are met:

- Sudden onset and rapid progression of symptoms
- Life-threatening Airway and/or Breathing and/or Circulation problems
- Skin and/or mucosal changes (flushing, urticaria, angioedema)

The following supports the diagnosis:

- Exposure to a known allergen for the patient

Remember:

- Skin or mucosal changes alone are not a sign of an anaphylactic reaction and these in isolation do not require admission unless other concerns
- Skin and mucosal changes can be subtle or absent in up to 20% of reactions (some patients can have only a decrease in blood pressure, i.e., a circulation problem)
- There can also be gastrointestinal symptoms (e.g. vomiting, abdominal pain, incontinence)
- Patients presenting with angioedema- check if they are on ACE I and stop.

EDU Plan (include how long should be observed):-

Management checklist

All patients should have:

Patent IV cannula	
Drug chart written, including own regular medication (if not implicated in the reaction)	
Regular antihistamines if continuing skin problems / itch or ongoing reaction. For tto - cetirizine OD for 3 days (less sedating than chlorpheniramine)	
Oral steroids if wheeze or severe reaction e.g. prednisolone OD for 3 days	
Salbutamol inhaler +/- neb if wheeze present or asthmatic	
Take blood sample for Mast Cell Tryptase and FBC. Mast cell tryptase should be repeated at 1-2 hours- please handover to named person on EDU if to be done on EDU and write this on EDU plan	

On discharge

All patients should have:

Referral to Allergy Clinic. Please email Patient name, DoB, S number and NHS number to RespiratoryAppointments@uhl-tr.nhs.uk Please give brief details of reason for referral, reaction, severity and trigger if known or attach the ICE discharge letter to email	
Two adrenalin self-administration devices prescribed on discharge (unless it is clear that the trigger is a drug that can be easily avoided in future)	
Discharge checklist has been completed	

Planned & agreed by

Referring doctor

Emergency Physician in Charge

Print names

Signatures

NB: Pathway Must be signed by both Emergency Physician in Charge

Patient details

Full
name

DoB

Unit
number

(use sticker if available)

Discharge checklist:

The following information should be given to the patients on discharge

Tick Box

1.	Information about anaphylaxis, including the signs and symptoms of an anaphylactic reaction	
2.	Information on what to do if an anaphylactic reaction occurs (use the adrenaline injector and call emergency services)	
3.	A demonstration of the correct use of the adrenaline injector and when to use it	
4.	Advice about how to avoid the suspected trigger (if known)	
5.	Information about the need for referral to a specialist allergy service and the referral process	
6.	Information about patient support groups.	

Name of person completed:-

Signature:-

Time & Date completed:-